



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه فرحات زينهم رفاعى

العنوان : ..

الحي / المدينة : الحوامديه

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 27710042102023 رقم التليفون : 01156528909

العمر (تاريخ الميلاد) : 04-10-1977 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____
التوقيع: (_____)

المقرر بما فيه : _____
التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة : _____
الوظيفة : _____
الوظيفة : _____

التوقيع : _____
التوقيع : _____
التوقيع : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المدونة في هذا النموذج صحيحة. رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. _____ /د. _____ /د. _____ /د. _____

قرار اللجنة

التشخيص النهائي : _____
خطة العلاج : _____
المستشفى : _____
جهة العلاج : _____
ملاحظات : _____

قسم : _____
نفقات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمة فرحات زينهم وفاء
National ID: 27710042102023



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

(271)

عاجل

جمهورية مصر العربية
وزارة الصحة والسكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة



م. ع. ع. ع.

خطاب إداري

الرقم القومي	٢٧٧١٠٠٤٢١٠٢٠٢٣
إسم المواطن	فاطمة فرحات زينهم رفاعي

رقم الطلب	٦٥٨٦٣٠٦٤	تاريخ اللجنة ٢٠٢٦.٠٥.١٥
الحالة	خطاب إداري	
نص الخطاب		جهة الإرسال جوسناف روسي انترناشيونال - مصر (EPE) *
(نوافي بنتيجة العينة)		
٢٦٧٦٢ / ١٤٧٤٣ / ٢٨٦٧٣		

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تحريراً في ٢٠٢٦-٠٥-١٥
٢٦٧٦٢ / ١٤٧٤٣ / ٢٨٦٧٣

Name: فاضله فرحات زينهم رفاعي
MR. : 10534
GUSTAVE / ROUSSY
since 1900

تقرير طبي

المريض

تحية طبية وبعد،،

بشأن تقرير طبي
للمريض

إيماء للطبيب المعتمد
من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالة

تاريخ وساعة : mm/dd/yyyy --:-- --
الخروج

تاريخ وساعة : mm/dd/yyyy --:-- --
الدخول

وبعد الكشف والفحوصات والمتابعة وجد انه

وزم سرطاني بالثدي اليمين

مقطعيه على الصدر و البطن و الحوض بالصبغة

تحريراً في .

التاريخ : mm/dd/yyyy

الوقت : --:-- --

محمد الشراوي

الطبيب المعالج :

يعتمد .

Patient Name الأستاذة / فاطمة فرحات زيتهم
Patient No. 27710042102023
Path No. 927/26
Sex Female
Age 48 Y
Referring Doc.

Request No. 13005764
Request Date 08/04/2026
Verification Date 21/04/2026
Financial Category جوسف روسي - مبادره
Department Out Patient

Registered by .marwa mohamed yous

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/04/2026 10:43:59

Page 2 of 2

Printed By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فاطمة فرحات زينهم
Patient No. : 27710042102023
Path No. : 927/26
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13005764
Request Date : 08/04/2026
Verification Date : 21/04/2026
Financial Category : جوفيف روسى - عيادته
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 7/4/2026, at 2:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

-Three fragmented grey white tissue cores, ranging in size between 0.5 cm & 1.3 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with focal lobular component.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3, with focal lobular component.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 28/04/2026 10:43:51

Page 1 of 1

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فاطمة فرحات زينهم

Patient No. : 27710042102023

Path No. : 927/26

Sex : Female

Age : 48 Y

Referring Doc. :

Request No. : 13005764

Request Date : 08/04/2026

Verification Date : 21/04/2026

Financial Category : جوسف - مبداء

Department : Out Patient

IHC No. :

Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass (BIRADS 4c) diagnosed "Invasive duct carcinoma grade 3 with focal lobular component".

Nature of Specimen: Tru-cut biopsy, collected on 7/4/2026, at 2:00 pm. Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 45%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3, with focal lobular component.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 45%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/04/2026 10:43:59

Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No, 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



١١/٥٦٥٢١٩.٩



NAME : Fatma Farahat Zeinhom

Date: 26/3/2026

BILATERAL DIGITAL MAMMOGRAPHY AND BREAST ULTRASOUND

Indications:

- Right breast mass

Technique:

- Bilateral CC and MLO views
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular densities (ACR B)
- Right breast showing dense well defined irregular lesion with spiculated margins .
- Bilateral axillary LNS more evident on right side.
- US revealed right breast upper inner quadrant showing an irregular hypoechoic heterogenous solid mass lesion with vascular intake on color doppler with spiculated margin and distorted surrounding parenchyma seen at 1:2 oclock measures about 3x3cm.
- Normal fibroglandular element of left breast with no definite masses detected..
- Normal nipple areola complex.
- Non specific axillary LNs are seen with preserved fatty hilum.

CONCLUSION :

- Right breast suspicious mass lesion as described for tissue biopsy (BIRAD IVc)

DR/Marina Maher

Patient's Name: Fatima faRahat Zeinhem
Date: 2026-06-06
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- o Right breast lesion measuring 53 x 36 mm for sono-mammography.
- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ Right breast lesion for sono-mammography.
- ❖ No pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

Ultrasound Guide 1 clip insertion:

Clinical summary: Pathologically proven right breast carcinoma a 3 cm mass at 2 o'clock position for clip insertion.

Technique:

- Under complete sterile conditions, One metallic clip is inserted within the center of large malignant mass lesion and measures about 3 cm and located at 2 o'clock position.
- This was confirmed by mammography CC & ML views.

Res. Regards

Dr. Imen Aïef
Breast Imaging consultant
El-Basssi Intervention

Name: Mrs. Fatma Farhat Zenhom

Date: 11/5/2026

ID: 2226016411

Acc.: 5253273

Referring Prof. Dr: Mohamed El-Nabarawy.

Tc-99m MDP whole body bone scan

Technique:

Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Typed Mah 27/05/76 5:45:10 PM

Issue No (1)

Page 1 of 1

Code: F/IRC-OP-20-01

مركز الطاهرة للطب الاشعاعي والتداليل الطبية
11/5/2026
ID: 2226016411
Acc.: 5253273
Referring Prof. Dr: Mohamed El-Nabarawy.

ACCH 00117
HOTLINE
19173

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CERTIFICATION
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CERTIFICATION

التداليل الطبية
مركز الطاهرة للطب الاشعاعي والتداليل الطبية
11/5/2026
ID: 2226016411
Acc.: 5253273
Referring Prof. Dr: Mohamed El-Nabarawy.

Name: Mrs. Fatma Farhat Zenhom

Date: 11/5/2026

ID: 2226016411

Acc.: 5253273

Referring Prof. Dr: Mohamed El-Nabarawy.

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Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Typed M sh 22050570 143107PM

Issue No. (1)

Page 1 of 1

Code: F/TRC-OP-20-01

مركز الطاهر للطب النووي والتحاليل الطبية
11/5/2026
تم إجراء فحص التصوير النووي للجسم بأكملها باستخدام Tc-99m MDP
بعد حقن 15 مللي من المتتبع في الوريد. تم الحصول على صور للجسم بأكمله
في اتجاهات الأمام والخلف. أظهرت النتائج توزيعاً متجانساً للمتتبع في
العظام مع وجود تغييرات تنكسية في الفقرات العنقية والظهرية والكبدية
والكليةتين المرئيتين.



HOTLINE
19173



مركز الطاهر للطب النووي والتحاليل الطبية
11/5/2026
تم إجراء فحص التصوير النووي للجسم بأكملها باستخدام Tc-99m MDP
بعد حقن 15 مللي من المتتبع في الوريد. تم الحصول على صور للجسم بأكمله
في اتجاهات الأمام والخلف. أظهرت النتائج توزيعاً متجانساً للمتتبع في
العظام مع وجود تغييرات تنكسية في الفقرات العنقية والظهرية والكبدية
والكليةتين المرئيتين.

NAME : Fatma Farahat Zeinhom

Date: 26/3/2026

**BILATERAL DIGITAL MAMMOGRAPHY AND BREAST
ULTRASOUND**

Indications:

- Right breast mass

Technique:

- Bilateral CC and MLO views
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular densities (ACR B)
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- Normal fibroglandular elemnt of left breast with no definite masses detected..
- Normal nipple areola complex..
- Non specific axillary LNs are seen with preserved fatty hilum.

CONCLUSION :

- Righ breast suspicious mass lesion as described for tissue biopsy (BIRAD IVc)

DR/Marina Maher

Echocardiography Report

Patient Name: Fatma Farahat Ref. By: Prof.Dr.: Date: 20.09.2026 Time: 8:41 AM

Echocardiographic Measurements (D and M-mode)

Measurement	Unit	Patient	Normal Range
LVEDD	Cm	5.2	3.5 - 5.8
LVESD	Cm	3.0	
SW	Cm	1.2	0.6 - 1.1
PW	Cm	1.2	0.6 - 1.1
FS	%	41%	25-40%
LA	Cm	4.3	2.0 - 4.0
Aorta	Cm	2.9	2.5 - 4.0
EF	%	72%	50-75%

Comment:

- Left ventricle:
 - Normal left ventricular internal dimensions with Normal LV systolic performance.
 - Pseudonormal pattern (Diastolic Dysfunction grade II)
 - No SWMA at rest. Mild Concentric LVH
- Left Atrium and Aortic root:
 - Dilated LA dimensions Normal Aortic root dimensions
- Right ventricle and atrium
 - Normal RV and RA dimensions with preserved RV global systolic function.
- Valvular assessment:
 - Mitral valve : Normal structure with Mild MR.
 - Aortic valve: Normal structure without significant gradient across the valve, Vmax=1.6 m/s.
 - Tricuspid valve: normal structure with Mild TR, ePASP=42mmHg
 - Pulmonary valve: normal structure and flow with normal MPT.
- Apparently Intact interatrial and interventricular septae by TTE.
- Normal pericardial thickness, no pericardial effusion.
- No intracardiac masses or thrombi by TTE.

CONCLUSION:

Normal LV dimensions and normal LV systolic function with Mild Concentric LVH

Diastolic Dysfunction grade II, Dilated LA
Mild MR, Mild TR, Mild Pulmonary Hypertension

Dr/Saif Alislam Agiz, Cardiology Consultant
I/uch obliged

اسم المريض بالإحاطية: Fatmah Farahat Zenhom	كود المريض: 99145
تاريخ التقرير: 2026/09/08 12:49:53 PM	السن: ٤٨ سنة ١١ شهر

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven right breast carcinoma 3 cm mass at 2 o'clock position for clip insertion.

Technique:

- Under complete sterile conditions, One metallic clip is inserted within the center of large malignant mass lesion and measures about 3 cm and located at 2 o'clock position.
- This was confirmed by mammography CC & ML views.

Best Regards

*Dr. Iman Adel
Breast Imaging consultant
& Breast Intervention*

Patient ID: 96405	Patient Name English: Fatimah Farahat Zenhom Refai
المرضى أ. ٩٦٤٠٥	20260821 10:59:29 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: a 43 years old female patient with pathologically proven IDC with focal lobular component grade III (-ve ER, -ve PR, +ve HER2 and Ki67= 45 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: C (scattered fibro-glandular densities)

RIGHT BREAST:

- Low energy (Non contrast) images: The right breast shows ill-defined dense mass lesion at the middle 1/3 UIQ With scattered foci of microcalcifications inside the lesion and extending beyond it measures collectively about 8x4.5x7.6 cm and about 5 cm from the nipple.
- Recombined (Post contrast) images: The right breast shows mild heterogeneous middle 1/3 LIQ mass enhancement measures about 5 x 4.5x 4.5 cm at its AP, TS and CC diameters with anterior linear non mass enhancement corresponding to microcalcifications seen measuring about 2 cm and about 5 cm from the nipple.
- Also a small enhancing focus seen anterosuperior to the index lesion about 3 cm away measuring about 7x3 mm
- Normal skin / nipple / areolar complex.
- Complementary US:
 - The UIQ breast shows ill-defined spiculated hypocochoic mass lesion located at clock position and measures about 3x2.5 cm and about 3 cm from the nipple
 - A 11 o'clock 3 CMFN small area of altered parenchymal echogenicity seen measuring about 7x3 mm(most likely corresponding to small enhancing focus)
 - No suspicious looking axillary LNs

LEFT BREAST:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show no pathological uptake.
- Normal skin / nipple / areolar complex.
- Complementary ultrasound:
 - Few enlarged axillary lymph nodes with diffuse cortical thickening

IMPRESSION:

RIGHT BREAST: (BIRADS 6= pathological proven) Right UIQ multifocal middle 1/3 pathologically proven IDC as described above

LEFT BREAST: (BIRADS 2= benign) Normal study

Best regards,

Dr. Norhan osama,MD

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
غريحات زينهم رفاعي
ش ٩ جسر المنوات
الحوامدية - الجيزة



٢٧٧ ١٠ ٠٤ ٢١ ٠٢ ٢٣

KF8749618



NAME : Fatma Farahat Zeinhom

Date: 26/3/2026

BILATERAL DIGITAL MAMMOGRAPHY AND BREAST ULTRASOUND

Indications:

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- Normal nipple areola complex.
- Non specific axillary LNs are seen with preserved fatty hilum.

CONCLUSION :

- Righth breast suspicious mass lesion as described for tissue biopsy (BIRAD IVc)

DR/Marina Maher

Patient's Name: Fatima faRahat Zeinhem
Date: 2026-06-06
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- o Right breast lesion measuring 53 x 36 mm for sono-mammography.
- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ Right breast lesion for sono-mammography.
- ❖ No pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

Patient Name : الأستاذ / فاطمه فرحات زبيهم
Patient No. : 27710042102023
Path No. : 927/26
Sex : Female
Age : 48 Y
Referreing Doc :

Request No. : 13005764
Request Date : 08/04/2026
Verification Date : 21/04/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 7/4/2026, at 2:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

-Three fragmented grey white tissue cores, ranging in size between 0.5 cm & 1.3 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with focal lobular component.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.

Diagnosis:

- Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3, with focal lobular component.
 - For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/04/2026 10:43:51

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name

الاستاذة / فاطمة فرحات زبيد

Patient No.

27710042102023

Path No.

927/26

Sex

Female

Age

48 Y

Referring Doc.

Request No. 13005764

Request Date 08/04/2026

Verification Date 21/04/2026

Financial Category

جوستاف روسي - مبادره

Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/04/2026 10:43:59

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Patient Name : الأستاذ / فاطمه فرحات زيتهم
Patient No. : 27710042102023
Path No. : 927/26
Sex : Female
Age : 48 Y
Referreing Doc. :

Request No. : 13005764
Request Date : 08/04/2026
Verification Date : 21/04/2026
Financial Category : جومستاف روسي - مبادره
Department : Out Patient
IHC.No. :
Registered by : .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass (BIRADS 4c) diagnosed "Invasive duct carcinoma grade 3 with focal lobular component".

Nature of Specimen: Tru-cut biopsy, collected on 7/4/2026, at 2:00 pm. Cold ischemia time is 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 45%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3, with focal lobular component.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 45%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

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Dr.Duaa Mohamed

Printing Date : 28/04/2026 10:43:59

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Name: Mrs. Fatma Farhat Zenhom

Date: 11/5/2026

ID: 2226016411

Acc.: 5253273

Referring Prof. Dr: Mohamed El-Nabarawy.

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

- No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel. MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Typed: M Sh 27/05/20 3:45:16 PM

Issue No (1)

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Code: F/IRC-OP-20-01

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T.C. 1100
Date: 11/5/2026
ID: 2226016411
Acc.: 5253273
Referring Prof. Dr: Mohamed El-Nabarawy.